

ABC INTERNATIONAL SCHOOL

Powai, Mumbai – Maharashtra

MEDICAL & HEALTH DOCUMENTS

Category 13 — Consent Forms · Vaccination Records · Health Screening · Illness Reporting · Medication Authorisation

Institution	ABC International School – Powai
Location	Hiranandani Gardens, Powai, Mumbai – 400076, Maharashtra
Dataset Category	13 — Medical / Health Documents
Applicable To	All Students (Nursery – Grade XII), Staff & School Nurse
Board	CBSE (Pre-Primary – XII) Cambridge IGCSE (Gr. 9–10)
Academic Year Ref.	2025–26
Document Generated	19 May 2026
Medical Staff	Full-time School Nurse Part-time Medical Officer (visiting — twice weekly)
Medical Room	Ground Floor, Admin Block Open: 7:00 AM – 5:30 PM (all school days)
Regulatory Basis	Clinical Establishments Act 2010 Maharashtra Public Health Act CBSE Health & Wellness Guidelines RTE Act 2009 POCSO Act 2012
Purpose	Student Health Administration, Legal Compliance, Chatbot Training & Audit Readiness

DISCLAIMER: This is a synthesised representative dataset for ABC International School, Powai, prepared for documentation, training and compliance-readiness purposes. All medical decisions must be taken by qualified medical professionals. Verify all protocols and drug information with the school's medical officer before implementation.

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1. Medical Consent Forms

Medical Consent Forms are mandatory for all enrolled students and must be submitted at the time of admission and renewed at the start of every academic year. These forms authorise the school nurse and medical officer to provide first aid and emergency medical treatment when parents cannot be reached immediately. All forms are stored in the student's individual health file in the Medical Room.

ABC INTERNATIONAL SCHOOL — POWAI

Hiranandani Gardens, Powai, Mumbai – 400076 | Medical Room Ext.: 104 | medical@ABC.org

MEDICAL CONSENT & EMERGENCY TREATMENT AUTHORISATION FORM

Form Reference: PIS/MED/MCF/2025–26/ ____ | Academic Year: 2025–26

PART A — STUDENT IDENTIFICATION

Student Full Name:	_____
Admission Number:	PIS/POW/____ / ____ Grade: ____ Section: ____
Date of Birth:	DD / MM / YYYY Age: ____ years
Gender:	Male / Female / Other
Blood Group:	A+ / A- / B+ / B- / O+ / O- / AB+ / AB- (Circle one)
Aadhaar No. (Child):	____ - ____ - ____
Academic Year:	2025–26
Class Teacher:	_____

PART B — EMERGENCY CONTACTS

Primary Contact Name:	_____
Relationship to Student:	Father / Mother / Guardian
Primary Mobile:	+91 _____
Alternate Mobile:	+91 _____
Office / Work Phone:	_____
Email Address:	_____
Secondary Emergency Contact Name:	_____
Relationship:	_____
Secondary Mobile:	+91 _____
Family Doctor / Paediatrician:	_____
Doctor's Clinic & Phone:	_____
Preferred Hospital (if any):	Hiranandani Hospital, Powai / Other: _____

PART C — KNOWN MEDICAL CONDITIONS & ALLERGIES

Condition	Yes / No	Details / Medication / Emergency Protocol
Severe Food Allergy (anaphylaxis)	Yes / No	Allergen(s): _____ EpiPen prescribed: Y / N EpiPen on campus: Y / N
Drug / Medication Allergy	Yes / No	Drug name(s): _____ Reaction type: _____
Asthma / Respiratory Condition	Yes / No	Inhaler type: _____ Dosage: _____ Trigger factors: _____
Epilepsy / Seizure Disorder	Yes / No	Last episode: _____ Rescue medication: _____ Protocol: _____
Diabetes (Type 1 / Type 2)	Yes / No	Type: ____ Medication: _____ Hypoglycaemia signs: _____
Cardiac / Heart Condition	Yes / No	Diagnosis: _____ Cardiologist: _____ Restrictions: _____
Sickle Cell / Blood Disorder	Yes / No	Condition: _____ Precautions: _____
Bee Sting / Insect Allergy	Yes / No	EpiPen prescribed: Y / N Other medication: _____
Latex Allergy	Yes / No	Reaction type: _____
Bleeding Disorder / Haemophilia	Yes / No	Type & factor level: _____ Emergency protocol: _____
Autism Spectrum Disorder (ASD)	Yes / No	Support triggers to avoid: _____ Calming strategies: _____
Any other significant condition	Yes / No	Describe: _____

PART D — CURRENT MEDICATIONS ON CAMPUS

If your child requires any medication during school hours, please complete this section AND submit a Medication Authorisation Form (Section 5 of this document) for each medication.

Medication Name	Dosage	Frequency / Timing	Condition It Treats	Stored In
				☐ Medical Room ☐ Student's bag
				☐ Medical Room ☐ Student's bag
				☐ Medical Room ☐ Student's bag

PART E — TREATMENT AUTHORISATION

I / We, the undersigned parent(s) / guardian(s) of the above-named student, hereby:

- **AUTHORISE** the School Nurse and/or visiting Medical Officer to administer first aid, over-the-counter medications listed in the school's approved formulary (paracetamol, ORS, antihistamine, antiseptic), and basic wound care as necessary when I/we cannot be reached immediately.
- **AUTHORISE** school staff to call emergency services (108 / ambulance) and transport my/our child to the nearest hospital in the event of a medical emergency requiring urgent treatment.
- **AUTHORISE** the hospital / emergency physician to provide necessary emergency medical treatment when I/we cannot provide consent in time due to circumstances beyond our control.
- **ACKNOWLEDGE** that school staff will make every reasonable effort to contact me/us before administering any treatment beyond basic first aid.
- **AGREE** to bear all costs of emergency medical treatment, ambulance and hospitalisation.
- **UNDERTAKE** to inform the school immediately of any change in my child's medical condition, medications, allergies or emergency contact details.

■ IMPORTANT: The school nurse does NOT administer prescription medications unless a completed Medication Authorisation Form is submitted with the original prescription. Prescription medications will not be given based on verbal instructions alone.

PART F — SPECIFIC CONSENT OPTIONS (Tick all that apply)

☐ First aid and wound dressing	☐ Oral paracetamol (fever / pain)
☐ ORS / oral rehydration solution	☐ Oral antihistamine (mild allergic reaction)
☐ Topical antiseptic / antibiotic cream	☐ Cold / ice pack application
☐ Calling 108 ambulance in emergency	☐ Transport to hospital by school vehicle
☐ Transport to hospital by ambulance	☐ EpiPen administration (if prescribed — see Part C)
☐ Inhaler administration (if prescribed — see Part C)	☐ Blood glucose check (if diabetic — see Part C)
☐ Annual health screening by school medical officer	☐ Vision / hearing screening
☐ BMI and growth measurement	☐ Dental check-up (if school dental camp organised)
☐ Deworming tablet (govt. programme)	☐ Iron & folic acid tablet (govt. programme — applicable grades)

PART G — DECLARATIONS & SIGNATURES

I / We declare that all information provided in this form is accurate and complete to the best of my / our knowledge. I / We understand that withholding medical information could endanger my child's health and that the school cannot be held liable for adverse outcomes arising from incomplete or inaccurate information provided by me / us.

Father / Guardian 1 Name:	_____
Father / Guardian 1 Signature:	_____ Date: ____ / ____ / 2025
Mother / Guardian 2 Name:	_____
Mother / Guardian 2 Signature:	_____ Date: ____ / ____ / 2025
Relationship to Student:	_____

FOR SCHOOL / MEDICAL ROOM USE ONLY

Form Received By:	Name: _____ Designation: _____
Date Received:	DD / MM / YYYY
Health File Created / Updated:	☐ Yes ☐ No — Reason: _____
Medical Room Register Entry:	Entry No.: _____ Date: _____
School Nurse Signature:	_____ Date: _____
School Medical Officer Review (if applicable):	_____ Date: _____

1B — Field Trip / Off-Campus Activity Medical Consent Form

This supplementary consent form is issued for each field trip, sports event, exchange programme or off-campus activity. It must be signed and returned at least 3 school days before the event. Students without this form will not be permitted to participate.

FIELD TRIP MEDICAL CONSENT & HEALTH DECLARATION

Student Name:	_____
Grade & Section:	Grade _____ Section _____ Adm. No.: _____
Event / Trip Name:	_____
Date(s) of Trip:	From: DD/MM/YYYY To: DD/MM/YYYY
Destination:	_____
Accompanying Teacher:	_____
Emergency Contact During Trip:	+91 _____ (parent must be reachable)
Is your child fit to participate?	☐ Yes, fully fit ☐ Yes, with restrictions (specify): _____ ☐ No
Any medical condition relevant to this trip?	☐ None ☐ Yes: _____
Medication to be carried on trip:	☐ None ☐ Yes — Medication Authorisation Form attached: ☐ Yes / ☐ No
Dietary restrictions (for catering):	☐ None ☐ Vegetarian ☐ Jain ☐ Allergy-based: _____ ☐ Other: _____
Swimming participation (if applicable):	☐ Permitted ☐ Not permitted ☐ N/A
I consent to my child participating in this trip:	☐ Yes ☐ No
Parent / Guardian Signature:	_____ Name: _____ Date: _____

2. Vaccination Records

Vaccination records are collected at the time of admission and updated annually. The school follows India's Universal Immunisation Programme (UIP) schedule and CBSE / Maharashtra Health Department guidelines. Parents must submit the child's immunisation card or a certificate from the paediatrician. Records are maintained in the student's health file.

2.1 Student Immunisation Record Card

STUDENT IMMUNISATION RECORD	
ABC International School – Powai Confidential Health Record	
Student Full Name:	
Admission No.:	Grade: Section:
Date of Birth:	DD / MM / YYYY Gender: M / F / Other
Paediatrician:	Phone:
Record Updated:	Date: By (Nurse):

SECTION A — UNIVERSAL IMMUNISATION PROGRAMME (UIP) VACCINES							
Vaccine	Dose s	Recommended Age	Date Given (Dose 1)	Date Given (Dose 2)	Date Given (Dose 3)	Booster / Dose 4+	Verified By
BCG (Tuberculosis)	1	At birth		N/A	N/A	N/A	
Hepatitis B	3	Birth, 6 wks, 6 months				N/A	
OPV (Oral Polio)	4+	6 wks, 10 wks, 14 wks + boosters					
IPV (Inactivated Polio)	2	6 wks, 14 wks			N/A	N/A	
DPT (Diphtheria-Pertussis-Tet.)	5	6,10,14 wks; 18 months; 5 yrs					
Hib (Haemophilus influenzae b)	3	6, 10, 14 weeks				N/A	
PCV (Pneumococcal)	3	6, 14 wks, 9 months				N/A	
Rotavirus	3	6, 10, 14 weeks				N/A	
Measles / MR	2	9–12 months; 16–24 months			N/A	N/A	
MMR (Measles-Mumps-Rubella)	2	12–15 months; 4–6 years			N/A	N/A	
Varicella (Chickenpox)	2	12–18 months; 4–6 years			N/A	N/A	
Hepatitis A	2	12–23 months; 6 months later			N/A	N/A	
Typhoid (injectable)	1+	2 years+; repeat every 3 years		N/A	N/A		

Vaccine	Dose s	Recommended Age	Date Given (Dose 1)	Date Given (Dose 2)	Date Given (Dose 3)	Booster / Dose 4+	Verified By
Td (Tetanus-Diphtheria booster)	1	10 years; 16 years		N/A	N/A		
Influenza (seasonal)	Annual	From 6 months; annual		N/A	N/A	Annual	
COVID-19 (as applicable)	2+	As per Govt. schedule			N/A		

SECTION B — ADDITIONAL / OPTIONAL VACCINES

Vaccine	Recommended For	Doses	Date Given (Dose 1)	Date Given (Dose 2)	Date Given (Dose 3)	Verified By
HPV (Human Papillomavirus)	Girls: 9–14 yrs (Grades 4–9); boys: optional	2 or 3				
Meningococcal	Travel to high-risk areas; outbreaks	1–2			N/A	
Japanese Encephalitis	As per regional risk; travel	2			N/A	
Rabies (pre-exposure)	High-risk exposure situations	3				
Dengue (Dengvaxia)	9–45 years; serostatus confirmed	3				
Pneumococcal (23-valent PPSV)	Children with chronic illness / asplenia	1–2		N/A	N/A	

SECTION C — SCHOOL-ORGANISED VACCINATION DRIVES

Programme	Target Grade	Vaccine / Supplement	AY 2025–26 Date	Consent Required	Records Updated By
Govt. School Immunisation Programme (SIP)	Grades 1, 5, 10	Td booster; MR booster (as per schedule)	September 2025	Yes — Form 1A Part F	School Nurse + BMC
Deworming Drive (National Deworming Day)	All Grades 1–12	Albendazole 400mg tablet	February 2026	Covered in Form 1A	School Nurse + BMC
Iron & Folic Acid (IFA) Programme	Grades 6–12	Weekly IFA tablet	Ongoing	Covered in Form 1A	School Nurse
HPV Vaccination Drive (State Govt.)	Grade 6 Girls	HPV vaccine Dose 1 & 2	As per BMC schedule	Separate consent form issued	School Nurse + BMC
Seasonal Influenza Camp	All Grades (opt.)	Injectable influenza vaccine	October 2025	Yes — separate form	Visiting Medical Officer
COVID-19 Booster (if applicable)	All eligible	As per Govt. advisory	As notified	Yes — separate form	School Nurse + BMC

Note: Parents who wish to opt OUT of any school-organised vaccination drive must submit a written letter to the Principal at least 5 school days before the scheduled drive, with a valid reason and their child's current vaccination certificate.

2.2 Class-wise Vaccination Coverage Summary Register

Grade	Section	Total Students	Vaccination Drive	Consents Received	Vaccinated	Opted Out	Coverage %	Nurse Signature
Grade 6	A	42	HPV Dose 1 (Girls)	21/21 girls	20	1	95.2%	
Grade 6	B	41	HPV Dose 1 (Girls)	20/20 girls	20	0	100%	
All	—	860	National Deworming	820	815	45	94.8%	
Gr. 1,5,10	—	200	Td Booster (SIP)	195	192	8	96.0%	

3. Health Screening Forms

Annual health screening is conducted for all students by the visiting Medical Officer and School Nurse. Screenings are scheduled in July–August for the current academic year. Parents are notified of results within 7 school days and referred to specialists where indicated. All records are confidential.

3.1 Annual Student Health Screening Record

ANNUAL STUDENT HEALTH SCREENING RECORD

ABC International School – Powai | AY 2025–26 | CONFIDENTIAL

Student Name:	_____
Admission No.:	_____ Grade: _____ Section: _____ Roll No.: _____
Date of Birth:	DD / MM / YYYY Age: _____ years _____ months
Date of Screening:	DD / MM / YYYY Time: _____ AM / PM
Screened By:	Dr. _____ Reg. No.: _____
Assisted By (Nurse):	_____

SECTION A — ANTHROPOMETRIC MEASUREMENTS

Parameter	Measurement	Normal Range (Age-based)	Status	Action Required
Height (cm)	_____ cm	Per CDC/IAP growth chart	Normal / Low / High	■ None ■ Refer paediatrician
Weight (kg)	_____ kg	Per CDC/IAP growth chart	Normal / Underweight / Overweight / Obese	■ None ■ Dietary counselling ■ Refer
BMI (kg/m²)	_____	18.5–24.9 (adults); age-specific chart for children	Normal / Low / High	■ None ■ Nutrition counselling ■ Refer
BMI-for-Age Percentile	_____ %ile	5th–85th = Healthy Weight	Healthy / Underweight / Overweight / Obese	■ Refer dietitian ■ Parent letter
Head Circumference	_____ cm	Age-appropriate (Pre-Primary only)	Normal / Abnormal	■ None ■ Refer neurologist
Arm Circumference (MUAC)	_____ cm	>12.5 cm = Normal (5 yrs+)	Green / Yellow / Red	■ None ■ Nutritional support

SECTION B — VITAL SIGNS

Parameter	Reading	Normal Range	Status	Action Required
Blood Pressure (mmHg)	_____ / _____	Age-appropriate (paediatric chart)	Normal / High / Low	■ None ■ Repeat ■ Refer cardiologist
Pulse / Heart Rate (bpm)	_____	60–100 bpm (adults); higher for children	Regular / Irregular	■ None ■ Refer
Temperature (°C)	_____ °C	36.1°C – 37.2°C	Normal / Fever / Low	■ None ■ Send home ■ Nurse monitoring

Parameter	Reading	Normal Range	Status	Action Required
SpO ₂ (Pulse Oximetry %)	_____ %	95–100%	Normal / Low	■ None ■ Urgent referral if < 90%
Respiratory Rate (/min)	_____	12–20 (adult); higher for children	Normal / Abnormal	■ None ■ Refer pulmonologist

SECTION C — VISION SCREENING

Test	Right Eye (OD)	Left Eye (OS)	Both Eyes (OU)	Normal?	Action
Visual Acuity (Snellen chart)	6 / _____	6 / _____	6 / _____	Y / N	■ None ■ Refer ophthalmologist
Colour Vision (Ishihara plates)	Pass / Fail	N/A	N/A	Y / N	■ None ■ Parent notification
Cover Test (squint/strabismus)	Normal / Abnormal	Normal / Abnormal	Normal / Abnormal	Y / N	■ None ■ Refer
Glasses / Contact Lenses worn	Y / N	Y / N	Y / N	N/A	■ Updated prescription recommended
Eye Redness / Discharge	Y / N	Y / N	Y / N	Y / N	■ None ■ Treat ■ Exclude if contagious

SECTION D — HEARING SCREENING

Test	Right Ear	Left Ear	Normal?	Action Required
Pure Tone Audiometry (whisper / tuning fork)	Pass / Refer	Pass / Refer	Y / N	■ None ■ Refer ENT / audiologist
Otoscopy (ear canal / eardrum inspection)	Normal / Abnormal	Normal / Abnormal	Y / N	■ None ■ Refer ENT
Hearing Aid in use	Y / N	Y / N	N/A	■ Classroom seating accommodation noted

SECTION E — DENTAL SCREENING

Parameter	Findings	Status	Action Required
Dentition (Primary / Mixed / Permanent)	_____	Normal / Abnormal	■ None ■ Refer dentist
Dental Caries (cavities)	None / Mild / Moderate / Severe	Normal / Abnormal	■ None ■ Refer dentist urgently
Gum Health (gingivitis / bleeding)	Normal / Mild / Moderate	Normal / Abnormal	■ None ■ Dental hygiene counselling
Malocclusion / Dental Alignment	Normal / Mild / Significant	Normal / Abnormal	■ None ■ Refer orthodontist
Oral Hygiene (plaque / tartar)	Good / Fair / Poor	Normal / Abnormal	■ Brushing technique taught

SECTION F — SKIN, HAIR & GENERAL EXAMINATION

Area	Findings	Normal?	Action Required
Skin (rashes, lesions, pallor, jaundice, cyanosis)	_____	Y / N	■ None ■ Treat ■ Exclude if contagious ■ Refer dermatologist
Hair & Scalp (lice, ringworm, alopecia)	_____	Y / N	■ None ■ Treatment advised ■ Exclude

Area	Findings	Normal?	Action Required
Lymph Nodes (cervical, axillary, inguinal)	Normal / Enlarged	Y / N	■ None ■ Refer
Abdomen (palpation — organomegaly)	Normal / Abnormal	Y / N	■ None ■ Refer
Spine (scoliosis / kyphosis screen)	Normal / Abnormal	Y / N	■ None ■ Refer orthopaedic
Feet / Flat Foot	Normal / Flat foot	Y / N	■ None ■ Physiotherapy referral
Nutritional Anaemia signs (pallor, fatigue)	Present / Absent	Y / N	■ None ■ Haemoglobin test ■ IFA supplement

SECTION G — MENTAL HEALTH & PSYCHOSOCIAL SCREEN

This section is completed by the School Counsellor in collaboration with the class teacher. Results are strictly confidential and shared only with the student's parents and relevant professionals. A 'flag' here is not a diagnosis — it indicates a need for further assessment.

Indicator	Observed (Y/N/NA)	Severity (Mild/Mod/Severe)	Counsellor Action
Persistent low mood / withdrawal			
Anxiety / excessive worry / school refusal			
Difficulty concentrating / learning			
Signs of bullying (victim or perpetrator)			
Possible self-harm indicators			
Poor peer relationships / social isolation			
Behavioural changes reported by teacher			
Sleep disturbance / fatigue (reported by parent)			

SECTION H — SCREENING SUMMARY & REFERRALS

Overall Health Status:	☐ Fit and Healthy — No concerns ☐ Mild concerns — monitoring recommended ☐ Referral required
Referrals Issued:	☐ Ophthalmologist ☐ ENT / Audiologist ☐ Dentist ☐ Paediatrician ☐ Dietitian ☐ Orthopaedic ☐ Dermatologist ☐ Neurologist ☐ Counsellor ☐ Cardiologist Other: ____
Parent Notification:	☐ Sent via SMS ☐ Sent via school app ☐ Physical letter Date: _____
PE Participation Status:	☐ Full participation ☐ Modified participation — restriction: _____ ☐ Excluded from PE
Special Accommodation Needed:	☐ None ☐ Preferential seating (vision/hearing) ☐ Dietary ☐ Other: _____
Follow-up Screening Date:	DD / MM / YYYY (if required)
Medical Officer Signature:	Dr. _____ Reg. No.: _____ Date: _____
School Nurse Signature:	_____ Date: _____

4. Illness Reporting SOP

This Standard Operating Procedure governs how illness — whether on campus or at home — is reported, assessed, managed and recorded at ABC International School, Powai. It covers routine illness, communicable disease outbreaks, and mass illness events. It applies to all students, staff and the school nurse.

4.1 On-Campus Illness — Step-by-Step Protocol

1

STUDENT REPORTS FEELING UNWELL

Student informs class teacher or any nearby adult. Class teacher does NOT dismiss the concern — treats every complaint seriously. Teacher completes a Sick Pass (pre-printed slip) and sends student to Medical Room. If student appears critically ill or injured: call Medical Room (Ext. 104) immediately — do NOT move student.

2

INITIAL NURSE ASSESSMENT IN MEDICAL ROOM

Nurse records: name, class, time, chief complaint, vital signs (temp, BP, SpO₂, pulse). Nurse consults student's health file for known conditions, allergies, medications. Student placed in rest area; monitored for 15–30 minutes for mild complaints. If symptoms resolve: student returns to class with nurse's note; entry made in Daily Illness Log. If symptoms do NOT resolve or are serious: proceed to Step 3.

3

PARENT / GUARDIAN NOTIFICATION

Nurse calls primary emergency contact immediately. If primary contact unreachable within 15 minutes: call alternate contact. SMS notification sent simultaneously through school communication system. Parent is advised to collect child or authorise school to send child home with escort. If parent authorises school transport: student escorted by female staff member to school gate / vehicle. If parent cannot come and no escort is available: student remains in Medical Room under nurse supervision.

4

EMERGENCY PROTOCOL — WHEN TO CALL 108

Call 108 IMMEDIATELY for: unconsciousness, severe difficulty breathing, anaphylaxis, seizure not resolving in 5 min, suspected fracture/head injury, chest pain, blood sugar < 60 mg/dL, SpO₂ < 90%, profuse uncontrolled bleeding. Simultaneously: notify Principal, call student's emergency contact. Nurse accompanies student in ambulance until parent arrives at hospital. Male staff to not accompany female students alone — female escort mandatory.

5

DOCUMENTATION

ALL illness visits — including minor ones — recorded in the Daily Illness Register (date, name, class, complaint, time in/out, action taken, follow-up). Serious incidents: complete Incident Report Form (Medical) within 24 hours. Monthly illness summary submitted to Principal by nurse on 1st of each month. Data used for health trend analysis: recurring complaints, seasonal patterns, class-wise illness clusters.

6

RETURN TO SCHOOL AFTER ILLNESS

Student absent for 1–2 days (mild illness): parent SMS confirmation sufficient. Student absent for 3+ days: medical fitness certificate from a registered MBBS doctor required before return. Specific conditions (see Table 4.2) require 'Fit-to-Return' certificate and may require a health check by school nurse on return. Student with active fever (> 38°C) must NOT be sent to school — will be sent home immediately if they arrive unwell.

4.2 Communicable Disease Exclusion & Return Policy

■ Parents are legally and ethically obligated to inform the school immediately if their child is diagnosed with any communicable disease listed below. Sending a child with an active communicable disease to school endangers the entire school community.

Disease / Condition	Exclusion Period	Return Condition	School Notification Required
Chickenpox (Varicella)	Until all lesions have fully crusted over (usually 5–7 days from onset)	Medical fitness certificate; all lesions crusted	Yes — class notified (no names); nurse monitors classmates
Measles	4 days after rash onset	Medical fitness certificate	Yes — BMC notified; school may be closed if outbreak
Mumps	5 days after swelling onset	Medical fitness certificate	Yes — BMC notified
Rubella (German Measles)	7 days after rash onset	Medical fitness certificate	Yes — especially important if pregnant staff present
Whooping Cough (Pertussis)	21 days from onset OR 5 days after start of antibiotic	Medical certificate + completion of antibiotic course	Yes — BMC notified
Hand, Foot and Mouth Disease	Until all blisters/sores have healed	Medical fitness certificate	Yes — class notified
Conjunctivitis (Pink Eye)	Until eye discharge has completely stopped	No discharge visible; no redness	Yes — hygiene advisory issued to class
Ringworm (Tinea)	Until treatment initiated + lesion covered	Treatment in progress; lesion covered with dressing	Inform parents of class for monitoring
Head Lice (Pediculosis)	1 school day (to allow treatment)	After treatment; nurse checks hair on return	Class-wide advisory sent; no names disclosed
Scabies	Until after first treatment	Completed treatment; no new burrows	Class-wide advisory; nurse examination
Impetigo	Until sores have healed / crusted OR 24 hrs after antibiotic	Medical certificate; sores covered	Hygiene advisory to class
Active Tuberculosis (TB)	Until declared non-infectious by treating physician (usually 2 weeks after effective treatment starts)	Specialist's fit-to-return certificate	Yes — BMC; school deep clean; contact tracing
Typhoid	Until afebrile AND medical clearance	Medical fitness certificate	Monitor classmates for symptoms
COVID-19 (if guidelines active)	As per current Govt. / BMC guidelines	Negative test or completion of isolation period per guidelines	As per BMC / MCGM protocol
Dengue / Malaria	No mandatory exclusion (not person-to-person); until afebrile	Medical fitness certificate; afebrile for 48 hrs	BMC fogging request made for campus
Any fever > 38°C (undiagnosed)	Until afebrile for 24 hours without fever-reducing medication	Afebrile for 24 hours; if > 3 days — medical certificate	Nurse monitors for clusters

4.3 Mass Illness / Outbreak Response Protocol

Trigger Threshold	Immediate Action	Escalation	Communication
3+ students in same class with same symptoms within 48 hours	Nurse reports to Principal; affected students isolated; Medical Room placed on alert	VP contacts BMC Health Officer; outbreak investigation begun	Parent advisory issued; class notified without disclosing names
10+ students school-wide with same symptoms within 72 hours	Principal activates Outbreak Response Plan; inform ABC Trust; consider partial closure	BMC / MCGM Public Health Department notified; independent investigation	School-wide parent communication; media queries directed to Principal only

Trigger Threshold	Immediate Action	Escalation	Communication
Any notifiable disease confirmed (BMC list)	Immediate BMC notification; affected area deep-cleaned; close contacts traced	BMC may order section/school closure; follow BMC directive	Direct parent notification; ABC Trust informed; transparent communication
Suspected food poisoning (5+ cases same day)	Remove all food; preserve samples for testing; contact food supplier; call BMC	BMC food safety team; hospital coordination	Immediate parent notification; full disclosure of action taken

4.4 Daily & Monthly Illness Reporting Register — Format

Date	Time	Student Name	Grade	Chief Complaint	Temp .	Action Taken	Parent Informed	Time Out	Follow-up
19/05/26	09:15	Ananya Iyer	5A	Headache / fever	38.2° C	Paracetamol; rest; parent called	Yes — 09:20	10:45 (parent pickup)	Fitness cert. required
19/05/26	11:30	Dev Sharma	7B	Abdominal pain	36.8° C	Rest 30 min; resolved; returned class	SMS sent	12:00 (returned class)	None
19/05/26	13:45	Ishaan Kulkarni	9A	Asthma — mild wheeze	36.9° C	Inhaler (own); rest; parent notified	Yes — 13:50	14:30 (parent pickup)	Specialist review advised

Note: Illness registers are reviewed monthly by the Medical Officer. Anonymised data is shared with the Principal and HOD for health trend planning. Original registers retained for 5 academic years.

5. Medication Authorisation Forms

■ **IMPORTANT POLICY:** The school nurse administers **ONLY** medications specifically authorised by a parent **AND** (for prescription drugs) supported by a valid prescription from a registered medical practitioner. **No medication — including paracetamol — may be given to a student based solely on verbal instruction from a parent. Self-medication by students is not permitted.**

5.1 School Approved Medication Formulary (Nurse-Administered — OTC Only)

The following over-the-counter medications may be administered by the school nurse based on Form 1A (Medical Consent Form — Part F) **WITHOUT** requiring a separate prescription, provided the consent box has been ticked by the parent. A separate Medication Authorisation Form (Form 5A) is still required for any medication **NOT** on this list.

Medication	Indication	Standard Dose (Nurse-Administered)	Max Frequency	Contraindications (Do NOT Give If)
Paracetamol (syrup / tablet)	Fever > 38°C; mild to moderate pain	10–15 mg/kg (children); 500–1000mg (adults/seniors)	Every 6 hours; max 4 doses / 24 hrs	Liver disease; paracetamol allergy; student already took dose at home
ORS (Oral Rehydration Salts)	Dehydration; diarrhoea; heat exhaustion	250–500 ml over 30–60 min; repeat as needed	As required	None (generally safe); do not use plain water for diarrhoea
Cetirizine (non-drowsy antihistamine)	Mild allergic reaction (hives, runny nose, itching)	0.25 mg/kg (children); 10mg (adults)	Once daily	Known antihistamine allergy; severe allergic reaction — use EpiPen instead
Antacid (Digene / Gelusil — liquid)	Mild indigestion; heartburn	10 ml (adults); 5 ml (6–12 yrs); do not give < 6 yrs	3 times daily (after meals)	Kidney disease; ongoing prescription antacid
Topical antiseptic (Betadine/Savlon)	Minor cuts, abrasions, wounds	Apply to clean wound; dress appropriately	As needed	Known iodine allergy (Betadine)
Topical antibiotic cream (Soframycin)	Minor infected wound; superficial abrasion	Thin layer on wound; cover with dressing	2–3 times daily	Known allergy to neomycin/framycetin
Cold/ice pack	Sprains; bruises; minor swelling	Wrapped ice pack; 10–15 min; no direct skin contact	As needed	Raynaud's disease; open wound

5A — Prescription Medication Authorisation Form

PRESCRIPTION MEDICATION AUTHORISATION FORM

One form per medication | ABC International School – Powai | Medical Room

Form Ref.: PIS/MED/MAF/2025–26/_____

SECTION A — STUDENT DETAILS

Student Full Name:	_____
Admission Number:	_____ Grade: _____ Section: _____
Date of Birth:	DD / MM / YYYY Age: _____
Blood Group:	_____ Weight: _____kg Known Drug Allergies: _____

Academic Year:	2025–26 Valid For: ☐ This academic year ☐ Specific period: _____
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SECTION B — MEDICATION DETAILS

Medication Name (generic):	_____
Brand Name (if specific):	_____
Strength / Concentration:	e.g., 500mg tablet / 125mg per 5ml syrup
Form:	☐ Tablet ☐ Capsule ☐ Syrup / Liquid ☐ Inhaler ☐ Injection (EpiPen) ☐ Eye drop ☐ Topical cream ☐ Other: _____
Dose to be Given at School:	_____ (e.g., 1 tablet / 5 ml / 2 puffs)
Time(s) to be Given:	☐ 8:00 AM ☐ 10:00 AM ☐ 12:30 PM (lunch) ☐ 2:30 PM ☐ Only if needed — condition: _____
Medical Condition Being Treated:	_____
Duration of Treatment:	From: DD/MM/YYYY To: DD/MM/YYYY ☐ Ongoing / Chronic
Prescription Attached:	☐ Yes — Dr.: _____ Dated: _____ ☐ No — OTC (approved list only)
Special Instructions:	e.g., Give with food / shake well / store in fridge / check blood sugar first: _____
Signs to Watch For (side effects):	_____
Action if Side Effect Observed:	☐ Call parent ☐ Call doctor ☐ Call 108 Details: _____

SECTION C — MEDICATION STORAGE

Storage Requirements:	☐ Room temperature ☐ Refrigerate (2–8°C) — Nurse fridge ☐ Protect from light ☐ Other: _____
Medication Supplied For:	_____ days / _____ doses (Parent must resupply before stock runs out)
Medication Handed To Nurse By:	Name: _____ Date: _____
Medication Received By Nurse:	Name: _____ Date: _____ Quantity: _____
Expiry Date on Medication:	MM / YYYY (Expired medication will NOT be administered — parent to replace)

SECTION D — EMERGENCY MEDICATION (EpiPen / Inhaler / Glucagon)

If your child has been prescribed an EpiPen, rescue inhaler or glucagon kit for emergency use, this section MUST be completed. Two devices should be provided — one kept in the Medical Room and one with the student (for Grades 6+) or with the class teacher (for Grades 1–5).

Emergency Medication Type:	☐ EpiPen / Epinephrine auto-injector ☐ Salbutamol inhaler (reliever) ☐ Glucagon kit ☐ Rectal diazepam ☐ Other: _____
Prescribed For:	_____ (Condition: anaphylaxis / severe asthma / hypoglycaemia / seizure)
Dose & Administration:	_____
When to Use (triggers):	_____

Administration Steps:	Confirm allergy/trigger → Administer (per device instruction) → Call 108 → Call parent
Device Location:	☐ Medical Room (primary) ☐ With student in bag ☐ With class teacher ☐ Both Medical Room + student
Expiry Date:	MM / YYYY (Parent must replace 1 month before expiry)
Staff Trained to Administer:	_____

SECTION E — PARENT / GUARDIAN AUTHORISATION

I / We, the parent(s) / guardian(s) of the above-named student, hereby authorise ABC International School, Powai's school nurse and designated staff to administer the medication described in this form in accordance with the instructions provided. I / We confirm that:

- The medication has been prescribed by a registered medical practitioner (for prescription drugs) and a copy of the prescription is attached.
- I / We have informed the school of all known allergies and current medications to avoid drug interactions.
- The information provided is accurate and I / We accept responsibility for any consequences arising from incomplete or inaccurate information.
- I / We understand that the school nurse will administer medication in good faith based on the information provided and is not liable for adverse reactions arising from undisclosed medical information.
- I / We will ensure continuous supply of medication and timely replacement of expired medications.

Parent / Guardian 1 Name & Signature	Parent / Guardian 2 Name & Signature	Date
_____	_____	_____

SECTION F — MEDICATION ADMINISTRATION LOG

Date	Time	Dose Given	Administered By	Student Response / Side Effects	Parent Notified	Nurse Signature
					Y / N	
					Y / N	
					Y / N	
					Y / N	
					Y / N	
					Y / N	
					Y / N	

Note: Administration log is reviewed monthly by the Medical Officer. Any adverse drug reaction is reported to the prescribing doctor and parents immediately. Medication records are retained for 5 years after the student leaves the school.

5B — Medication Return / Disposal Form

Student Name & Grade:	_____
Medication Name:	_____
Reason for Return:	☐ Treatment completed ☐ Medication changed ☐ Student left school ☐ Medication expired ☐ Other: _____
Quantity Remaining:	_____units / ml / doses

Returned To Parent:	Name: _____ Date: _____ Signature: _____
Disposed (if expired):	Method: _____ Date: _____ Nurse Signature: _____
Nurse Closing Note:	_____

END OF DOCUMENT — ABC International School, Powai | Medical & Health Documents | Generated: 19 May 2026
Synthesised representative dataset. All medical decisions must be made by qualified medical professionals. Verify all drug dosages, protocols and regulatory requirements with the school's medical officer before implementation. | medical@ABC.org | +91 22 2570 0123 Ext. 104